SACTCHECK · REGIMEN-SPECIFIC SACT CONSENT

Cisplatin + Etoposide

PROTOTYPE · GOVERNANCE REVIEW REQUIRED

Extensive-stage small-cell lung cancer · chemotherapy · NCCP 00280 · version 7 · 21-day cycle

Patient name	DOB	Hospital no.	Date

Treatment context
Cisplatin + Etoposide is chemotherapy used for extensive-stage small-cell lung cancer. It aims to control the cancer and its symptoms. Ask your oncology team what benefit is realistic for you and how this option compares with other suitable treatments.

Schedule
Cisplatin is planned on Day 1. Etoposide is planned on Days 1–3: usually all three days by infusion, or a prescribed alternative with an infusion on Day 1 and capsules twice daily on Days 2–3. The source plans 4–6 cycles, usually every 21 days. The team may delay, reduce or stop treatment according to your health and response.

1. TREATMENT DISCUSSION

- ☐ Purpose, expected benefit and alternatives explained
- ☐ Indication, medicines, sequence and duration confirmed
- ☐ Blood tests and any individually indicated heart checks discussed
- ☐ Support medicines and phase changes explained
- ☐ Fertility, contraception and breastfeeding discussed where relevant
- ☐ Common, serious and lasting drug-specific risks discussed below
- ☐ Urgent symptoms and local contact details confirmed

2. REGIMEN / AGENT-SPECIFIC MATERIAL RISKS

Etoposide
TOPOISOMERASE II CHEMOTHERAPY

- ☐ Low blood counts, infection and bleeding
- ☐ Nausea, vomiting and fatigue
- ☐ Hair loss and mouth soreness
- ☐ Allergic or infusion reactions
- ☐ Effects on fertility and harm to an unborn baby
- ☐ Diarrhoea and liver-test changes
- ☐ Low blood pressure during infusion
- ☐ A later blood cancer (leukaemia)

Cisplatin
PLATINUM CHEMOTHERAPY

- ☐ Low blood counts, infection and bleeding
- ☐ Nausea, vomiting and fatigue
- ☐ Hair loss and mouth soreness
- ☐ Allergic or infusion reactions
- ☐ Effects on fertility and harm to an unborn baby
- ☐ Kidney injury and electrolyte loss
- ☐ Hearing changes or tinnitus, which may persist
- ☐ Nerve symptoms, which may persist

Discuss the individual aim, expected benefit and uncertainty, alternatives including not proceeding, serious and lasting risks, kidney/hearing/nerve injury, infection/bleeding, fertility and pregnancy advice, infusion versus capsule logistics, delays and dose changes, and support at home. Confirm that any additional immunotherapy or radiotherapy has its own consent discussion.

Individual aim / benefit, alternatives, material risks and questions:

3. CONSENT CONFIRMATION

- ☐ Expected benefit, important risks and reasonable alternatives discussed
- ☐ Patient questions invited and answered; patient-specific material risks / priorities considered
- ☐ Emergency contact / toxicity advice and written information offered where appropriate
- ☐ Patient understands consent is voluntary and may be withdrawn

Patient / person giving consent _____ Date _____

Clinician obtaining consent _____ Date _____

Prototype only. This one-page form is a regimen-specific consent aid and requires formal governance, consultant and oncology-pharmacy review before clinical use. The current NCCP regimen, local/national consent policy, current medicine information, patient-specific material risks and clinical judgement remain authoritative.
Base regimen: NCCP 00280 v7.

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